



LEGAL SUPPORT HELP

CASE MANAGEMENT SUPPLEMENT

ADVANCED PROTOCOLS: PACKAGING AUDITS, SUBROGATION CUTS, DV & CMS ANALYTICS

This advanced operational supplement introduces critical strategic dimensions to training case managers. By implementing rigorous verification checks across medical packaging indexing, statutory subrogation enforcement matrices, hidden secondary vehicle losses, and Case Management System (CMS) data-stale analytics, the firm insulates its files from automated carrier mitigation models and ensures optimized recovery streams.

1. The Demand Package Audit & Exhibit Indexing Standards

Insurance evaluation engines (such as Colossus) systematically penalize unstructured evidence portfolios. If clinical charts contain disjointed timelines, mixed history streams, or unsubstantiated invoices, the evaluation software automatically drops the global claim multiplier. Case Managers must enforce an absolute primary source presentation standard:

- **The Chronological Tab Method:** Sort and bind records into distinct, chronologically sequential PDF Exhibits following the natural recovery timeline: Segment A: Scene Logs/EMS/ER Charts; Segment B: Primary Care/Conservative Chiropractic Notes; Segment C: Advanced Injections/Interventional Pain Management; Segment D: Specialist/Orthopedic Surgical Consultations; Segment E: Primary Source Billing Ledgers and First-Party PIP Summaries.
- **Redacting Non-Accident Material:** Defense counsel can weaponize unrelated medical disclosures discovered during pre-suit discovery. Case Managers must audit charts line-by-line to execute legal redactions of sensitive, non-accident related entries (e.g., prior family histories, historical mental health entries completely unrelated to the traumatic mechanisms, or alternative physical care streams outside the current injury timeline) while ensuring compliance with state discovery codes.
- **The One-to-One Matching Check:** Every single line-item charge on a provider's balance ledger must tie directly to a matching clinical evaluation report or diagnostic



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entry. If a diagnostic center invoices for multiple specialized views (e.g., continuous cervical sequences) but the localized radiologist report only reads a basic film profile, you must secure the supplemental report or demand an updated, clean ledger from the facility before submission. This prevents adjusters from dropping unproven special damages.

2. Advanced Subrogation Negotiation Strategies (ERISA vs. Medicare)

Squeezing third-party liens and dynamic statutory subrogation lines dictates the ultimate net recovery realized by the client. Case Managers must operate with advanced contractual and statutory enforcement tactics to push back on reimbursement assertions:

A. Dismantling ERISA Self-Funded Plan Assertions

Private healthcare administrators frequently assert absolute reimbursement rights over third-party settlements by claiming absolute federal preemption under ERISA. Do not accept a generic lien summary sheet as validation of an enforceable claim. Execute this strict review sequence:

- 1. Demand Primary Documentation:** Issue a formal written demand to the plan administrator requesting the complete Summary Plan Description (SPD), the master legal Plan Document, and the latest Form 5500 filing records.
- 2. Audit Subrogation Enforceability:** Review the master legal text line-by-line. If the plan language lacks explicit, bulletproof language establishing a 'First-Priority Right of Recovery' over third-party proceeds, or if it lacks a clear 'Rejoinder/Abolishment of the Made-Whole Doctrine,' the administrator cannot legally claim 100% recovery out of a compromised third-party settlement pool.

B. Leveraging State Equitable Protections & Portal Adjustments

- **The Made-Whole Shield:** When a recovery plan has solid baseline language, you must apply the Made-Whole and Common Fund Doctrines. Argue that if a client was forced to accept a restricted, low third-party bodily injury policy limit that failed to completely clear their total economic and non-economic losses, the plan must proportionally reduce its demands because the victim was never truly 'made whole' by the tortfeasor.
- **Medicare Portal Audits:** For federal statutory lines, utilize the Medicare Secondary Payer Portal (MSPRP) to actively file administrative 'Disputes.' Audit the itemized



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conditional payment profile line-by-line. If Medicare accidentally swept in routine maintenance prescriptions, unrelated primary care visits, or chronic conditions (e.g., maintenance hypertension or diabetes therapies) that happened to fall inside the accident window, execute a dispute action to drop those entries from the final recovery lien matrix.

3. Managing Property Damage Diminished Value & Loss of Use Claims

When a vehicle's property damage claim is integrated directly into a global demand package, Case Managers should look beyond basic body shop repair blueprints to capture the latent secondary financial losses tied to the client's property assets.

A. Diminished Value (DV) Recovery Protocols

Even if a vehicle is flawlessly restored to pre-collision structural parameters by an authorized repair facility, its baseline retail market valuation drops automatically because it now carries a permanent 'Carfax Accident History Profile.' This represents a real property loss.

- **Action Parameter:** If the client operates a late-model, high-equity vehicle with zero historic structural losses, instruct them to secure a certified Diminished Value Appraisal Report from an independent automotive engineer.
- **Demand Framework:** Add this calculated structural loss figure as a distinct property damage special damage line item inside your demand letter to force the carrier to pay for the true, long-term asset damage.

B. Loss of Use Calculations

The tortfeasor is legally liable for the loss of a client's vehicle use during the entire repair process. Case Managers must track this timeline to calculate property damage value:

1. **Map the Storage and Repair Window:** Calculate the exact number of days that elapsed from the split-second of the collision until the vehicle was fully repaired and returned, or until a formal total loss payout was completed.
2. **Apply Market-Rate Multipliers:** Multiply that complete day span by the daily commercial rental vehicle rate for that vehicle class in your local market. Note that under standard insurance rules, this claim is completely valid even if the client didn't end up renting a car or relied on family transport.



4. Advanced CMS Data Analytics for Stale Case Progression Tracking

Case Managers must use the Case Management System (CMS) as a predictive tool to catch internal performance blocks before they delay case lifecycles.

A. The 'Stale Status' Weekly Tracking Loop

Files frequently stagnate when transitioning between different status tags due to vendor delays or unreturned adjuster calls. Implement a strict weekly tracking review:

- **Execute Stale Filters:** Run a weekly administrative report tracking files flagged in Pending Demand or Demanded statuses.
- **The 21-Day Trigger:** If any file displays a zero-touchpoint index (meaning zero phone logs, incoming documents, or system touches) for more than 21 consecutive days, it must trigger an automatic level-1 alert. Case Managers must immediately make direct contact with the vendor or adjuster to push the file into negotiation or active escalation.

B. Managing High-Value LOP Treatment Inflation

Unmonitored treatment pipelines can allow non-party medical costs to spin out of control, creating a situation where outstanding provider liens exceed the available insurance policy caps. To prevent this, implement the following automatic safeguard tracking system:

Lien Exposure Ceiling	System Tracking Action Required	Escalation & Resolution Protocol
Accumulated Balance Exceeds \$15,000	The moment an unmonitored Letter of Protection (LOP) treatment log reaches \$15,000, trigger an automated task flag on the CMS dashboard.	Halt additional non-emergency care referrals. Request immediate interim ledger sheets and medical narratives from the clinic to verify ongoing clinical necessity.
Lien Profile Reaches 50% of Policy Cap	Calculate total outstanding medical balances against the confirmed policy limits (e.g., if medical bills hit \$12,500 against a \$25,000 BI boundary).	Generate a mandatory 'Policy Ceiling Warning.' Flag the file for the handling attorney to evaluate whether to stop care or prepare for immediate policy limit demand filing.



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Lien Exposure Ceiling	System Tracking Action Required	Escalation & Resolution Protocol
Lien Profile Reaches 75%+ of Policy Cap	Total medical bills approach or match the known limits of all available insurance policies combined.	Route the file directly to the firm's litigation division. Prepare the file to bypass normal pre-suit negotiation steps, as a formal lawsuit will be necessary to settle the case safely.